

3rd week Sunday case scenario

- 1- Dr.
- 2- Dr.
- 3- Dr.
- 4- Dr.

CASE 8

Mrs Ibtihal is a 19-year-old married student, who is referred to the Gynaecology admissions unit because of a 24-hour history of severe right iliac fossa pain. She is on Depot-Provera for contraception, and has as a result been amenorrhoeic for the past 18 months. On admission her BP is 100/65 mm Hg, her pulse is 104 bpm and her temperature is 37.1°C. On physical examination she appears in pain, despite having been given intramuscular Opiates on arrival. Her abdomen is generally tender with maximum tenderness in the right lower abdomen; there is marked guarding with rebound tenderness. Pelvic examination reveals severe cervical excitation, and right adnexal tenderness.

8.1. List the four most relevant differential diagnoses, and describe the salient presenting features and examination findings for each of these:

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8.2.Suggest the first line investigations that would help you narrow down your list of differentials.

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8.3.For each of the differential diagnoses, consider the longer term impact on the patient:

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Further investigations confirm the diagnosis of a torted right ovarian cyst, and the patient undergoes a laparoscopic oophorectomy.

8.4. Draw on available resources to explore causes of adnexal masses; consider the types of ovarian cysts likely to present in the reproductive age-group (including functional ovarian cysts).

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